

DefVetBridge Veterans Guide

Finding your information, organising your evidence,
and getting your claim ready.



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What DVB is, and is not. DVB is an independent tool that helps you organise your own medical and service evidence. It does **not** diagnose you, give legal or medical advice, or predict whether DVA will accept a claim. You and DVA make every real decision. When DVB says evidence "supports" a factor, that describes your documents, never an outcome.

DefVetBridge is an independent tool supporting the Australian veteran community. It is not affiliated with, or endorsed by, the Department of Veterans' Affairs.

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1. Getting into DVB

DVB is **invite only**. Someone from DVB (or your advocate) adds your email address, and that is what lets you in. There is no password to remember.

- Open the sign-in link in your invite email, or go to <https://dvm.defvetbridge.com/account/login>.
- Choose **Google** or **Microsoft**, and use the account that has **the same email address you were invited on**. A different address will be refused.
- The first time you sign in, your invite is matched to you. After that you just sign in the same way.
- No invite email? Check your junk folder, then ask DVB to **re-send it**: admin@defvetbridge.com.

What you can see

You see **only your own account**: your files, conditions and claims. An advocate can see only the veterans who have linked to them.

2. Finding your information

A claim is built on two kinds of information: **your own records** (what happened to you) and **the official rules** (what DVA must be satisfied of). This section is a map of both.

Your own records

- **Medical records**: request copies from every GP, specialist, hospital and imaging provider you have used. Ask for the full record, not a summary.
- **Service records**: postings, dates, deployments and any incident or exposure reports. These drive which rules apply to you.
- **DVA correspondence**: past claim decisions, reference numbers and letters. Keep every one, including rejections.
- **Anything in your own words**: dates, places and what happened, written down while you still remember. Statements from people who were there can matter.

Records from AFTER you discharge

Your service records show what happened to you. DVA also needs to see what your condition is **now**, so records made **after** you left the ADF matter just as much. Do not stop at your service file:

- **Specialist reports**: a recent report from the specialist who treats the condition (orthopaedic surgeon, psychiatrist, audiologist and so on), covering the diagnosis, how it affects you now and, where they can say it, its cause.
- **Your GP's complete patient record**: the whole file from every GP you have seen since discharge, not a short summary or a referral letter. Ask the practice for a full copy of your patient record.
- **Hospital, imaging and pathology results** from after discharge: scans, operation reports, discharge summaries.
- **Allied health notes** (physio, psychology, pharmacy lists) where they show ongoing treatment.

Ask for these early. Practices can take weeks, and a specialist may need an appointment to write a current report.

The official rules and guides

These are the documents DVA and the Repatriation Medical Authority use. Links were checked on 06/10/2026 unless marked.

Resource	What it is, in plain English	Where
Statements of Principles (SOPs)	For each condition, the factors that link it to service. Read the <i>medical definition</i> and the <i>factors that must exist</i> . You generally need to meet one factor for a condition to be linked. Each SOP is set out under a standard of proof (see below).	rma.gov.au/sops
Guide to Determining Impairment and Compensation 2026 (GARP M)	How permanent impairment is assessed under the MRCA. In force since 25 May 2026, and DVA says it is the only tool used from 1 July 2026. Older versions (the 2016 guide) are no longer in force.	legislation.gov.au/F2026L00595
Advocate guide and handbook	DVA's handbook on what advocates do and the support they can give. Useful if you want to know what to ask for.	dva.gov.au/sites/default/files/2023-04/atdp-rc-handbook.pdf (2023 edition)
Hearing services (accepted hearing loss)	DVA's page on hearing aids, tinnitus and assistive devices (Hearing Services Program, 1800 500 726). Covers Gold Card holders and White Card holders with accepted hearing loss or tinnitus.	dva.gov.au/what-we-help-with/health-support/hearing-services

The two standards of proof

- **Reasonable Hypothesis (RH):** the lower bar. It applies to certain kinds of service: warlike, non-warlike, operational, nuclear-test and hazardous service.
- **Balance of Probabilities (BoP):** the higher bar. It applies to peacetime service.
- Your service dates and type of service decide which one applies. DVB reads this from your Military History.

Which scheme applies changed on 1 July 2026. New claims lodged on or after that date are assessed under the **MRCA**, whenever and wherever you served. Older VEA and DRCA claims are no longer open to new lodgements. The scheme is decided by your **lodgement date**, not your service dates. Check DVA's website (dva.gov.au) for the latest detail, as the rules are still bedding in.

Finding the right SOP for a condition

- Search the condition name on rma.gov.au/sops. The same condition may have more than one SOP (for example, by type or by injury versus disease).
- Read the factors. Note which ones your own records could support, and which would need more evidence.
- In DVB, the **Conditions** and **Claim Readiness** pages do this matching for you and show the gaps. Always read the SOP yourself; DVB points you to it, it does not replace it.

3. Getting your documents in

DVB can only work with evidence it can see. Use the **Medical Files** page to upload files from your computer or phone: PDFs, Word documents and images.

- Upload everything, even if it looks unimportant. A small note can be the missing link.
- Once uploaded, DVB reads each document automatically: conditions, providers, dates and the type of document. A large scanned PDF can take several minutes.
- If a file shows an error or looks stuck, the **Dashboard** lists indexing problems with Retry and Skip buttons.

- Your original upload is never changed. DVB keeps it as you gave it.

4. Checking your details and conditions

Member Details and **Military History** hold your core profile and your postings and exposures. Other features, including form pre-fill, read from them, so check them early.

Conditions is the heart of DVB. For each condition it shows the evidence it found. Over time you will:

- **Combine duplicates** (the same condition worded two ways) with the merge tool.
- **Confirm or override SOP matches.** If a match looks wrong to you, say so. DVB is only as accurate as the corrections you give it.
- **Set claim status:** not yet claimed, part of a pending claim, accepted, or treatment-only. This reflects reality; DVB never decides it for you.
- **Link evidence** to the factors it supports.

Discovery suggests conditions you might not have thought of: ones the law recognises as flowing from a condition already accepted, and ones your own evidence supports but you have not claimed. Treat each as "worth asking my doctor or advocate about". Nothing is added without your yes.

5. Finding gaps before you lodge

The **Claim Readiness Report** (from a condition page) lists, factor by factor, what the SOP asks for and whether you have a document for it. Most claims that go wrong are incomplete, and this page exists to show you the gap while there is still time to fill it.

The **News** page checks weekly for new, changed or revoked SOPs and calls out any that touch your conditions. A red dot in the menu means there is something to look at.

6. Forms and your claim pack

- The **Forms** page pre-fills the right DVA form from your own profile and condition data, so you do not re-type your details. DVB prepares the form for the scheme that applies on the date you lodge.
- Anything only a treating clinician can complete (diagnosis, consultation dates, examining doctor, signature) is **left blank on purpose**. DVB never fills it in.
- A **claim pack** brings your form, your evidence list and your supporting statement together. Its wording is assembled from your stored records, not written by AI, because you sign it.
- **Take the pack to your GP or treating specialist.** Ask them to complete the medical section of each D2049 and to add a current report. Page 2 of every pack explains this in plain English.
- **Read every field yourself before you sign.** DVB never lodges anything. You print, sign and lodge it the normal way.

7. The usual order, and getting help

1. Sign in, then upload your documents and let indexing finish.
2. Check Member Details and Military History are accurate.
3. Review Conditions: confirm SOP matches, merge duplicates, link evidence.
4. Look at Discovery for anything you have not claimed.
5. Run the Claim Readiness Report before lodging anything.
6. Generate your form and claim pack, review every field, sign and lodge it yourself.
7. Record the claim on the DVA Claims page, and keep an eye on News.

You do not have to do this alone. An advocate or lawyer can work alongside you, and DVB can link them to your account so they see what you see, only with your approval. Questions about DVB itself: admin@defvetbridge.com.

If this process is weighing on you. **Open Arms** (openarms.gov.au) provides free counselling for veterans and families, and **Lifeline** is available any time on **13 11 14**.

Sources: Repatriation Medical Authority (rma.gov.au/sops); Guide to Determining Impairment and Compensation 2026 (legislation.gov.au/F2026L00595); DVA, Permanent impairment compensation (dva.gov.au/permanent-impairment-compensation). Rules change: check the official site before relying on any date or threshold in this guide.